

C790

Foundations in Nursing Informatics

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

On a cardiac step-down unit with a new coffee machine, a nurse notices a client's blood pressure has fallen steadily over four hours while heart rate has risen, and she recognizes the pattern as consistent with early sepsis. The unit's census is 24. Which DIKW level describes recognizing the pattern and what it means?

- A. Wisdom.
- B. Knowledge.
- C. Information.
- D. Data.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Using her understanding of sepsis trends, a nurse decides to call the rapid response team now rather than wait for the next set of vital signs. Which DIKW level is this?

- A. Data.
- B. Knowledge.
- C. Wisdom.
- D. Information.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A health system with three campuses is adding a barcode medication administration system. The vendor has a strong sales team. Which role is the informatics nurse best suited to fill?

- A. Translate nursing workflow needs to IT and the vendor, and involve frontline nurses in design and testing.
- B. Let the vendor design workflows because of its strong sales team.
- C. Make all final decisions alone to keep the project fast.
- D. Write the system's software code.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

After go-live, a medical-surgical unit with a new break room finds that nurses override 94% of drug-interaction alerts, most of them low severity. The hospital upgraded its Wi-Fi last month. What is the best informatics response?

- A. Upgrade the Wi-Fi again.
- B. Add more alerts to catch additional errors.
- C. Turn off all drug-interaction alerts.
- D. Review and tune the alerts so only high-severity ones interrupt, and remove or soften low-value ones.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

Which alert is most appropriate to keep as an interruptive hard stop?

- A. Every alert, since each one prevents an error.
- B. A reminder to confirm the client's preferred name.
- C. A notice that a lab value is barely outside the normal range and unchanged from yesterday.
- D. An order for penicillin for a client with documented penicillin anaphylaxis.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

Two hospitals that share a regional trauma network want to compare nursing outcomes, but one documents "skin breakdown" and the other "pressure injury," both in free text. Both use the same EHR vendor. What best enables meaningful comparison?

- A. Map documentation to a standardized, coded terminology such as SNOMED CT.
- B. Use CPT billing codes to record the outcome.
- C. Agree in a memo to use the same free-text phrase.
- D. Nothing more; the same EHR vendor already makes the data comparable.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

On a busy oncology unit, nurses print extra barcode labels and scan them at the nurses' station instead of at the bedside, because scanners often lose connection in patient rooms. The unit won a teamwork award last year. What is the best interpretation?

- A. The teamwork award shows the practice is safe.
- B. The workaround is fine because it saves time.
- C. The workaround signals a system problem; fix the bedside connectivity with nurses while stopping the unsafe practice.
- D. The nurses are simply noncompliant and should be disciplined.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A health system with 12 clinics is choosing how to launch a new EHR. Leaders want to limit risk and learn from early problems before full rollout. The CFO prefers a spring launch. Which go-live strategy fits best?

- A. Launch in spring, since timing is the main risk factor.
- B. A phased or pilot rollout starting with one or two clinics.
- C. Run the old and new systems side by side indefinitely.
- D. A big-bang launch at all 12 clinics on one day.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A team compares medication error rates after go-live with the baseline it collected before launch. Which project phase is this?

- A. None; once live, the project is finished.
- B. Implementation.
- C. Design.
- D. Evaluation.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A unit manager at a hospital that recently repainted its lobby must move nurses from paper flowsheets to electronic charting. Staff say the paper system works fine. Using Lewin's model, what should happen first?

- A.** Refreeze new habits with updated policies.
- B.** Unfreeze: share why the change is needed, such as error data, and involve staff.
- C.** Train everyone on the new system right away.
- D.** Mandate the switch, since resistance always fades.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

Six months after go-live, a manager updates policies, celebrates success, and removes the old paper forms so staff do not slip back. Which Lewin stage is this?

- A.** Refreezing.
- B.** Unfreezing.
- C.** Changing (moving).
- D.** Bargaining.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A nurse at a large academic medical center with a famous football program learns that a local celebrity is admitted to another unit. She is curious but not involved in the care. The EHR lets her open the chart. What should she do?

- A.** Open only the demographics page.
- B.** Open it, because public figures give up privacy rights.
- C.** Not open the chart, because access is limited to job-related need.
- D.** Open it, since the EHR allows it.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

On a cardiac step-down unit ~~with a new coffee machine~~, a nurse notices a client's blood pressure has fallen steadily over four hours while heart rate has risen, and she recognizes the pattern as consistent with early sepsis. ~~The unit's census is 24.~~ Which DIKW level describes recognizing the pattern and what it means?

Target word: DIKW level **Decides:** Synthesizing related information into a recognized pattern.

A	Confusion set	Wisdom is deciding what to do; she has not acted yet.
B	Correct	Linking trends into a meaningful pattern is knowledge.
C	Adjacent category	Information is data with context, such as one labeled reading; the pattern goes further.
D	Dropped element	Drops the interpretation; data are single raw values.

2. Answer C Concept 1 · transfer

Using her understanding of sepsis trends, a nurse decides to call the rapid response team now rather than wait for the next set of vital signs. Which DIKW level is this?

Target word: reverse: action given, name the level **Decides:** Applying knowledge to choose an action.

A	Inverted operation	The lowest level; this is the highest.
B	Confusion set	Knowledge is recognizing the pattern; the decision is the next level.
C	Correct	Wisdom is the judgment to act on knowledge.
D	Adjacent category	Information is contextualized data, not a decision.

3. Answer A Concept 2

A health system ~~with three campuses~~ is adding a barcode medication administration system. ~~The vendor has a strong sales team.~~ Which role is the informatics nurse best suited to fill?

Target word: informatics nurse role **Decides:** Bridging clinical workflow and technology.

A	Correct	The informatics nurse links practice and technology.
B	Camouflage	Sales strength does not make the vendor the expert on local nursing workflow.
C	Absolute language	Informatics work is collaborative; excluding users hurts adoption.
D	Adjacent category	Programming is a developer role.

4. Answer D Concept 3

After go-live, a medical-surgical unit ~~with a new break room~~ finds that nurses override 94% of drug-interaction alerts, most of them low severity. ~~The hospital upgraded its Wi-Fi last month.~~ What is the best informatics response?

Target word: best response **Decides:** Mostly low-value alerts are being overridden.

A	Camouflage	Network speed does not cause overrides.
B	Inverted operation	More alerts deepen alert fatigue.
C	Absolute language	Removes the high-severity protections too.
D	Correct	Reducing noise restores attention to the alerts that matter.

5. Answer D Concept 3 · transfer

Which alert is most appropriate to keep as an interruptive hard stop?

Target word: reverse: pick the high-value alert **Decides:** Severe, likely harm.

A	Absolute language	Treating all alerts as hard stops causes alert fatigue.
B	Confusion set	Useful but low risk; better as a passive prompt.
C	Adjacent category	Low clinical significance; interrupting adds fatigue.
D	Correct	High severity and high likelihood of harm justify interrupting.

6. Answer A Concept 4

Two hospitals ~~that share a regional trauma network~~ want to compare nursing outcomes, but one documents “skin breakdown” and the other “pressure injury,” both in free text. Both use the same EHR vendor. What best enables meaningful comparison?

Target word: what enables comparison **Decides:** Different free-text words for the same concept.

A	Correct	Codes give the same concept one meaning across systems.
B	Confusion set	CPT codes describe billable procedures, not nursing outcomes.
C	Adjacent category	Free text is still hard to compute and drifts over time.
D	Camouflage	Same software does not mean same terms or codes.

7. Answer C Concept 5

On a busy oncology unit, nurses print extra barcode labels and scan them at the nurses’ station instead of at the bedside, because scanners often lose connection in patient rooms. ~~The unit won a teamwork award last year.~~ What is the best interpretation?

Target word: meaning of the workaround **Decides:** Scanners fail at the bedside.

A	Camouflage	An award does not decide medication safety.
B	Dropped element	Drops bedside identification, the safety step barcoding exists for.
C	Correct	Workarounds reveal design gaps; the root cause must be fixed.
D	Confusion set	Blaming staff ignores the failing technology.

8. Answer B Concept 6

A health system with 12 clinics is choosing how to launch a new EHR. Leaders want to limit risk and learn from early problems before full rollout. ~~The CFO prefers a spring launch.~~ Which go-live strategy fits best?

Target word: go-live strategy **Decides:** Limit risk and learn before full rollout.

A	Camouflage	The season does not decide the rollout strategy.
B	Correct	Early sites expose problems before the whole system depends on it.
C	Adjacent category	Parallel runs are costly and should be short, not open-ended.
D	Inverted operation	Maximizes risk; problems hit everyone at once.

9. Answer D Concept 6 · transfer

A team compares medication error rates after go-live with the baseline it collected before launch. Which project phase is this?

Target word: reverse: activity given, name the phase **Decides:** Measuring results against a baseline.

A	Absolute language	Evaluation after go-live is a required phase.
B	Adjacent category	Implementation is the go-live itself.
C	Confusion set	Design builds the solution before launch.
D	Correct	Evaluation measures outcomes against goals and baseline.

10. Answer B Concept 7

A unit manager ~~at a hospital that recently repainted its lobby~~ must move nurses from paper flowsheets to electronic charting. Staff say the paper system works fine. Using Lewin's model, what should happen first?

Target word: first step **Decides:** Staff do not yet see a need to change.

A	Inverted operation	Refreezing is the final stage, not the first.
B	Correct	Readiness must be built before moving to the new system.
C	Dropped element	Skips unfreezing; resistant staff may not adopt it.
D	Absolute language	Resistance does not reliably fade without engagement.

11. Answer A Concept 7 · transfer

Six months after go-live, a manager updates policies, celebrates success, and removes the old paper forms so staff do not slip back. Which Lewin stage is this?

Target word: reverse: activity given, name the stage **Decides:** Locking in the new behavior.

A	Correct	Stabilizing the new way of working is refreezing.
B	Inverted operation	Unfreezing is the first stage.
C	Adjacent category	The transition already happened.
D	Confusion set	A stage from a grief model, not Lewin.

12. Answer C Concept 8

A nurse at a large academic medical center ~~with a famous football program~~ learns that a local celebrity is admitted to another unit. She is curious but not involved in the care. The EHR lets her open the chart. What should she do?

Target word: correct action **Decides:** She has no job-related need.

A	Adjacent category	Any unneeded access is a privacy violation.
B	Confusion set	Every patient has the same privacy protections.
C	Correct	System access is not permission; need-to-know applies.
D	Dropped element	Drops the need-to-know rule.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	5	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	8	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	DIKW hierarchy	Data are raw facts; information is data given context; knowledge is recognizing patterns and relationships; wisdom is using knowledge to decide and act.	Calling pattern recognition wisdom, or acting on a hunch information.	Is the nurse recording, interpreting, synthesizing, or deciding?
2	Role of the informatics nurse	The informatics nurse bridges clinical practice and technology, translating workflow needs and involving frontline users.	Thinking the role is writing code or letting the vendor design workflow.	A new system is coming and someone must represent nursing.
3	Clinical decision support and alert fatigue	Too many low-value alerts cause overrides; tune alerts so only high-severity ones interrupt.	Adding more alerts to improve safety.	Very high override rates.
4	Standardized terminologies	Coded terminologies such as SNOMED CT, LOINC, and nursing languages let data be shared and compared across systems.	Assuming the same vendor or agreed free text makes data comparable.	Two sites describe the same thing in different words.
5	Workarounds as signals	Workarounds point to system or workflow design problems; find the root cause with users while stopping unsafe practice.	Treating workarounds only as staff noncompliance.	Staff bypass a safety step because the tool does not work where they do.
6	Project life cycle and go-live strategy	Plan, analyze, design, implement, evaluate; a phased or pilot go-live limits risk, and evaluation compares results to a baseline.	Big-bang launch when leaders want low risk; skipping evaluation.	“Learn from early problems” or “compare to baseline.”
7	Lewin’s change model	Unfreeze (build readiness), change (move), refreeze (lock in new habits).	Training before people see why change is needed.	Staff say the old way works fine.
8	Privacy and need-to-know access	Access records only as your job requires, even when the system technically allows more.	Believing system access equals permission.	A curious clinician not involved in care.

Course scope (public description): This course addresses the integration of technology to improve and support nursing practice. It provides nurses with a foundational understanding of nursing informatics theory, practice, and applications. Topics include the role of nursing in informatics; use of computer technology for clinical documentation, communication, and workflows; problem identification; project implementation; and best practices.